

Clostridioides difficile Infection (CDI) — Clinical One-Pager

Facilitator quick reference · TeachIM (Zhang, Sept 2021), optimized Jun 2026 · CC BY-NC 4.0 · clinician-review before use

Test ONLY symptomatic patients (≥3 unformed stools/24 h or ileus). Colonization is common (~21% inpatients) — **do NOT test asymptomatic patients and do NOT test for cure.**

Diagnosis (2-step)

Test	Detects / performance
GDH Ag	Enzyme from ALL strains; high sens (94–96%)
Toxin A/ B	Toxigenic strains; high spec (94–98%)
NAAT	Toxin gene; confirms toxigenic — use for discordant results

GDH+ / toxin+ → CDI · both – → r/o · discordant → NAAT.
No repeat test <7 d.

Severity Classification

Class	Definition
Non-severe	Absence of severe criteria
Severe	WBC >15k OR Cr ≥1.5 mg/dL
Fulminant	Severe + hypotension/shock OR ileus/megacolon
Recurrent	Sx + positive test within 8 wk (~20%)

RF: age >65, healthcare exposure, antibiotics; community-acq: IBD, cardiac dz, CKD.

Treatment

Scenario	Regimen
Non-severe / severe (initial)	Fidaxomicin 200 mg BID ×10 d (preferred) or PO vanc 125 mg q6h ×10 d
Fulminant	PO vanc 500 mg q6h (PR if ileus) + IV metronidazole 500 mg q8h; surgery consult
1st recurrence	Fidaxomicin (pref) or tapered/pulsed PO vanc
≥2 recurrences	Tapered/pulsed vanc, vanc → rifaximin, or FMT/microbiota — refer

Metronidazole PO only if low-risk non-severe & neither agent available.

Recurrence Prevention & Precautions

- **Microbiota therapy (post-course):** Rebyota (rectal, 2022), Vowst (oral spores, 2023), or FMT.
- **Bezlotoxumab discontinued Jan 2025** — no longer available.
- High-risk on systemic abx: consider PO vanc 125 mg daily ppx (continue 5 d after abx).
- Contact + enteric precautions until 48 h after diarrhea resolves.
- **NOT recommended:** probiotics; stopping PPIs for prevention.

Pearls

- A positive test ≠ infection — symptoms make the diagnosis.
- IV vancomycin does NOT treat CDI (no luminal delivery).
- No test-of-cure: shedding persists ~56%; post-infectious IBS up to 25%.
- Community-acquired CDI (~40%) is rising — consider it in younger patients.